

Emergency and Specialist Medicine

Acute hyperkalaemia in adults

Use in the ED and inpatients areas for patients with K⁺ 5.5 or above

DO NOT use in

- Diabetic ketoacidosis; follow DKA guideline (B66/2011)
- Dialysis patients with elevated K⁺ levels but NO hyperkalaemic ECG changes immediately prior to a dialysis session
- Pts sent to ED from primary care; separate guideline (C41/2020)

Disclaimer:
This is a clinical template; clinicians should always use judgment when managing individual patients

Developed by Martin Wiese · Version 76

Re-approved by UHL PGC on 13Nov25
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Patient details

Full name

DoB

Unit number

(use sticker if available)

① Hyperkalaemic ECG changes?

☐ Yes - one or more of the below

Peaked T waves
(narrow base, high amplitude, sharp pointy apex and usually symmetrical)

Absent or flattened P waves

Wide QRS (>0.12sec)

Ventricular tachycardia (VT)

Merging S and T ('sine') waves

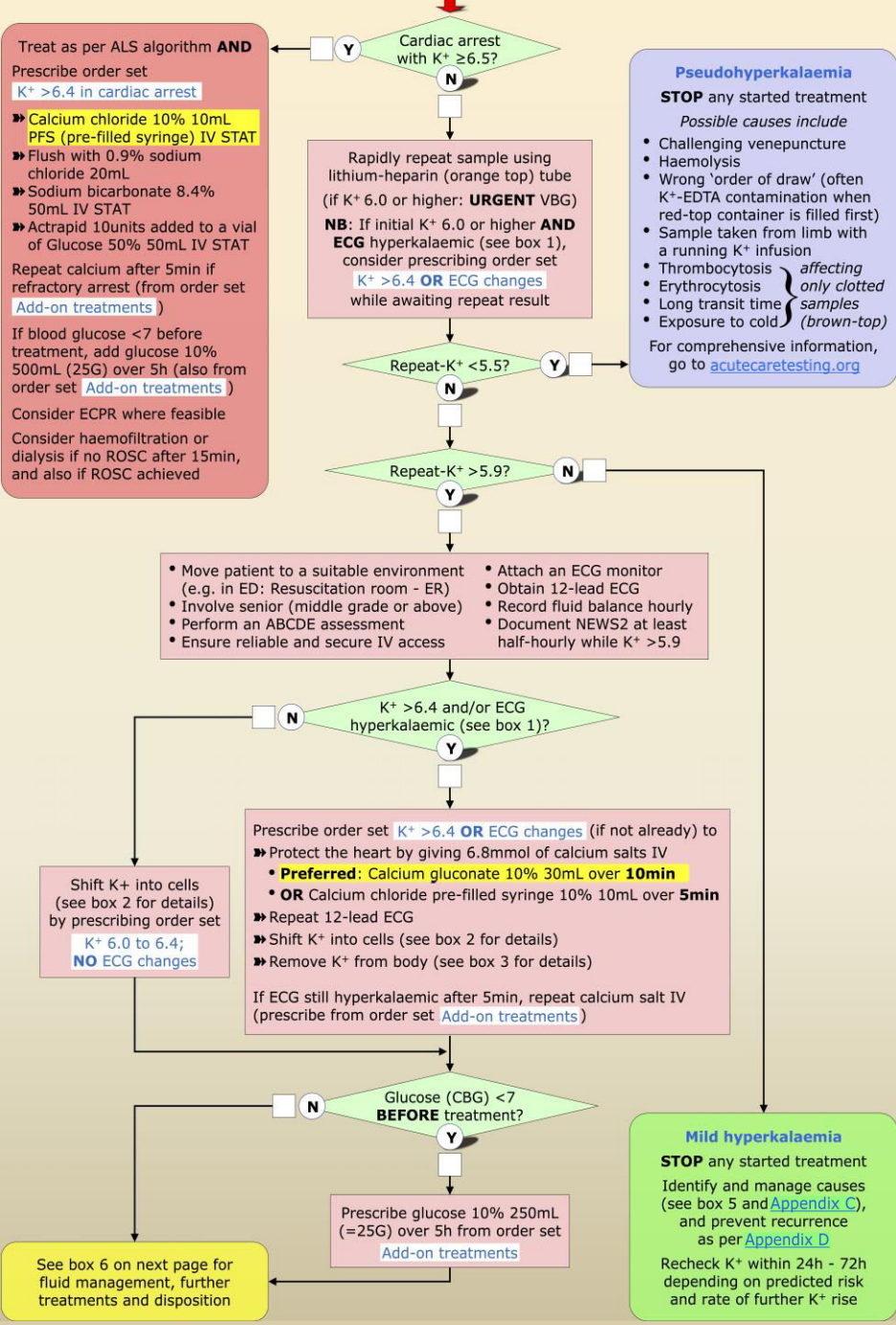
Bradycardia (sinus / AV block)

Pseudo-STEMI

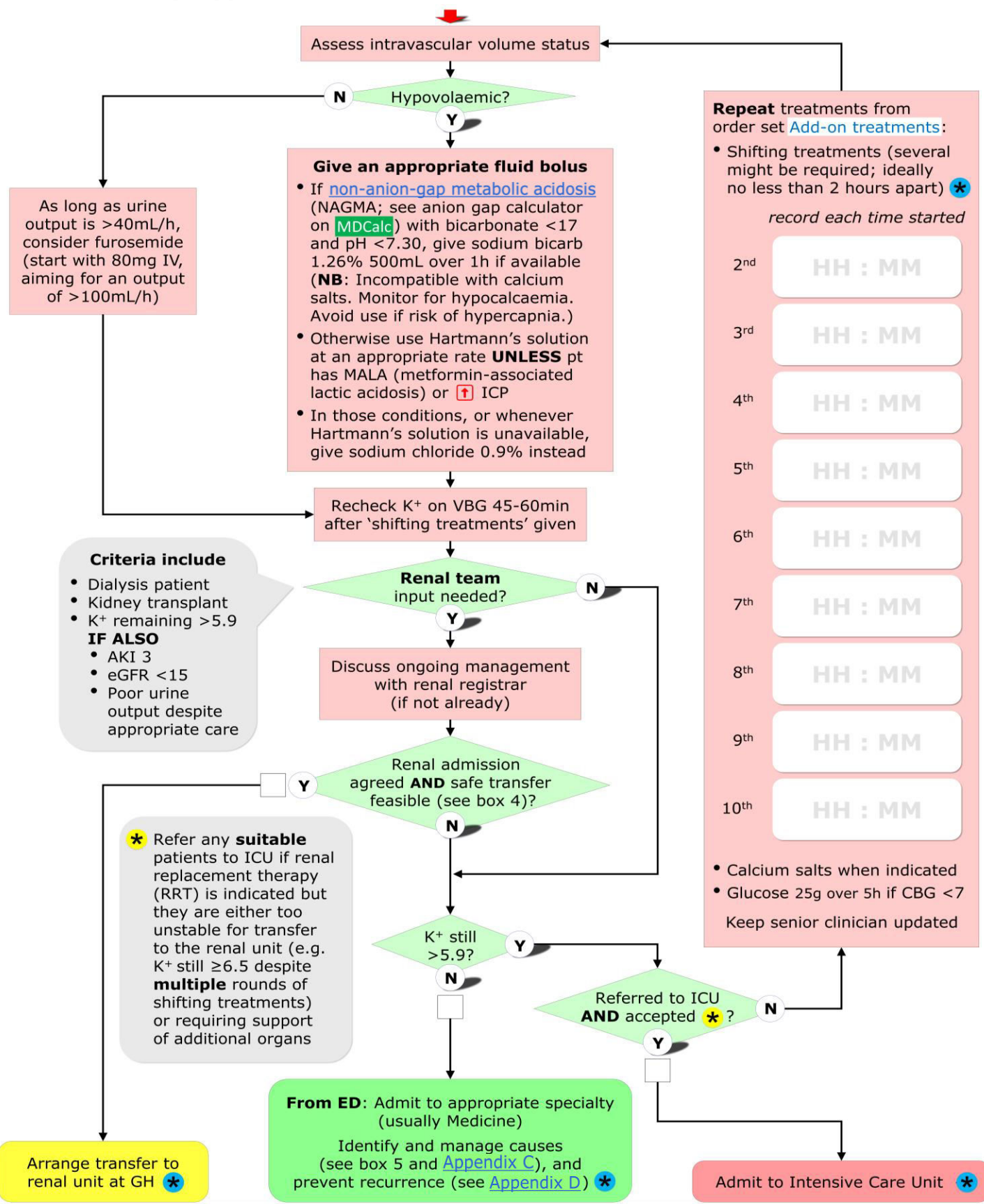
Brugada phenocopy

☐ No - none of the above

To prescribe any of the relevant medicines in NC Meds, go to Medical Emergencies > Hyperkalaemia (in the ED, go to Emergency Medicine (ED) > Common scenarios (ED) > Hyperkalaemia)



⑥ Emergency hyperkalaemia treatment continued



*** Monitoring**

- Blood glucose after each round of shifting treatments: After start every 30min for 2h, then hourly for another 4h
- K⁺ at least 1, 2, 4, 6 and 24 hours after last shifting treatment commenced

Patient was managed by

Print name

Signature

Role